METROPOLITAN INTERNAL MEDICINE CLINICAL RECORD

Patient Name: Harold Jenkins | Age: 71 | Sex: Male | UHID: HJ-MED-2026-1001
Attending Physician: Dr. Sarah Vance, MD

CHIEF COMPLAINTS:

Routine 6-month chronic disease follow-up for hypertension, hyperlipidemia, and benign prostatic hyperplasia. Reports mild morni

PRIMARY DIAGNOSIS:

1. Essential Hypertension (Moderate, Stage 2).
2. Primary Hyperlipidemia (Elevated LDL cholesterol).
3. Benign Prostatic Hyperplasia (BPH) with lower urinary tract symptoms.
4. Bilateral Knee Osteoarthritis.

CLINICAL INVESTIGATIONS & LAB RESULTS:

- Blood Pressure: 138/84 mmHg | Pulse Rate: 68 bpm
- Fasting Blood Sugar: 98 mg/dL | HbA1c: 5.7%
- Serum Creatinine: 1.0 mg/dL (eGFR 78 mL/min) | Serum Potassium: 4.6 mEq/L
- Total Cholesterol: 196 mg/dL | LDL: 114 mg/dL | HDL: 44 mg/dL | PSA: 2.4 ng/mL

DISCHARGE MEDICATIONS & PRESCRIPTION:

1. Tab. Lisinopril 20 mg - 1 tablet orally once daily morning (ACE Inhibitor for BP).
2. Tab. Atorvastatin 20 mg - 1 tablet orally daily at bedtime (Statin for lipids).
3. Tab. Tamsulosin 0.4 mg - 1 tablet orally daily 30 minutes after dinner (Alpha-1 blocker for BPH).
4. Tab. Paracetamol 500 mg - 1 tablet orally every 8 hours as needed for joint pain.
5. Tab. Cholecalciferol (Vitamin D3) 1000 IU - 1 tablet daily with breakfast.

CONTRAINDICATIONS & MEDICATION ALERTS:

- STRICTLY AVOID NSAIDs (Ibuprofen, Naproxen) with Lisinopril as they impair renal perfusion and raise blood pressure.
- ORTHOSTATIC HYPOTENSION ALERT: Lisinopril + Tamsulosin can cause dizziness upon standing. Sit on bed edge before ris

DIETARY & LIFESTYLE GUIDANCE:

- Low sodium DASH diet (<2g salt/day).
- Hydrate with 1.8 Liters water daily; limit evening fluids after 8 PM.
- Walk 25 minutes daily on flat ground.